

Follow-up Fetal Echocardiography

Sonographer + physician cheat sheet for a dedicated cardiovascular reassessment

CORE RULE: A follow-up fetal echo reassesses a previously evaluated fetal cardiovascular condition. It is not simply another look at the heart during an obstetric ultrasound.

1 Qualification gate

- ☐ **Target:** What specific cardiac lesion, functional issue, rhythm problem, or conduction risk is being reassessed?
- ☐ **Baseline:** Was a complete fetal echocardiogram previously performed? Record date, location, and major findings.
- ☐ **Question:** What must today's study decide: resolved, stable, improved, progressed, or new finding?

No prior complete fetal echo available? Do not automatically label today's examination as follow-up. Determine whether an initial complete fetal echocardiogram is appropriate.

2 Common follow-up targets

- Septal defect
- Pericardial effusion
- Valve stenosis or regurgitation
- Cardiomyopathy or hypertrophy
- Abnormal chamber size/function
- Outflow or arch abnormality
- Suspected coarctation
- Arrhythmia
- Conduction abnormality
- Known congenital heart disease

Other defined cardiovascular abnormalities may also require focused reassessment.

CODE THE COMPONENTS ACTUALLY PERFORMED AND SUPPORTED

76826
FOLLOW-UP 2D

Dedicated 2D fetal cardiac reassessment with stored diagnostic imaging and lesion-specific evaluation.

76828
SPECTRAL DOPPLER

Requires actual stored spectral waveforms. Color images alone do not support this component.

93325
COLOR FLOW

Color-flow velocity mapping when performed, stored, interpreted, and appropriately reported.

ORDER

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ACQUIRE

>

INTERPRET

>

DOCUMENT

>

CODE

All five should tell the same clinical story.

Acquire the Clinical Answer

Store diagnostic cine, color, and spectral evidence that directly addresses the target condition

A. 2D / GRAYSCALE | 76826

- ☐ Save **dynamic cine loops**, not only still images.
- ☐ Fetal position and situs, as appropriate
- ☐ Four-chamber view; LVOT; RVOT
- ☐ Three-vessel / 3VT views, as appropriate
- ☐ Aortic arch and ductal arch
- ☐ Relevant venous anatomy, when indicated
- ☐ Ventricular size, symmetry, and contractility
- ☐ Lesion-specific views and measurements

Clinical endpoint: Show the target abnormality well enough to determine whether it has resolved, remained stable, improved, or progressed.

C. SPECTRAL DOPPLER | 76828

Acquire and store actual waveforms when this component will be reported. Select sites based on the lesion and clinical question.

- | | |
|--|--|
| ☐ Mitral + tricuspid inflow | ☐ Aortic + pulmonary outflow |
| ☐ Aortic arch | ☐ Ductal arch |
| ☐ Pulmonary veins | ☐ Ductus venosus |
| ☐ Abnormal jet or vessel | ☐ Mechanical PR, when indicated |

Velocity accuracy: Align the Doppler beam as close to the direction of flow as possible. When accurate velocity measurement is needed, ASE advises an angle preferably less than about 20 degrees.

NO STORED SPECTRAL WAVEFORM = NO 76828
Color Doppler alone does not support spectral Doppler coding.

B. COLOR FLOW | 93325

- ☐ Interrogate the clinically relevant septa, valves, outflows, arches, shunts, and veins.
- ☐ Show flow direction, turbulence, acceleration, and regurgitation where relevant.
- ☐ Optimize box size, scale/PRF, gain, and frame rate for the fetal heart.
- ☐ Store representative color clips or images and describe the result in the report.

COLOR IS NOT SPECTRAL. A color box does not create a velocity waveform.

ALWAYS ASSESS

- | | |
|----------------------------------|-----------------------------------|
| • Heart rate + rhythm | • Semilunar valves, when relevant |
| • Chamber size + proportionality | • Direction of flow/shunting |
| • Ventricular function | • Cardiovascular deterioration |
| • AV valve function | • Hydrops, when appropriate |

Focused exam, complete clinical context.

Lesion-Specific Reassessment

Acquire what the lesion demands, then document anatomy, function, hemodynamics, rhythm, and interval change

LESION-SPECIFIC PROMPTS

PERICARDIAL EFFUSION

- Measure consistently; compare with prior
- Function + rhythm
- AV valve regurgitation
- Other fluid collections / hydrops

VSD / SEPTAL DEFECT

- Location + size
- 2D visualization + color flow
- Ventricular proportionality
- Outflows + arches as appropriate

VALVE DISEASE

- Valve morphology + color
- Spectral velocity when indicated
- Degree of regurgitation
- Ventricular size/function + progression

CARDIOMYOPATHY / HYPERTROPHY

- Septal + wall thickness
- Chamber size + systolic function
- AV inflow / outflow Doppler as relevant
- Effusion + hydrops assessment

ARRHYTHMIA / CONDUCTION

- Rate + rhythm documentation
- Atrial-ventricular relationship
- M-mode and/or Doppler correlation
- Mechanical PR when indicated; function + hydrops

PHYSICIAN: DEDICATED FOLLOW-UP REPORT

WHY?

State the specific reason for today's fetal cardiac reassessment.

BASELINE?

Identify the prior complete fetal echo: date, location, and relevant findings.

PERFORMED?

List only today's actual components: 2D, M-mode, color, spectral Doppler, measurements.

FOUND?

Describe relevant anatomy, function, hemodynamics, rhythm, and measurements.

CHANGED?

Compare directly: resolved, stable, improved, progressed, or new finding.

REPORT STARTER

"Follow-up fetal echocardiogram for reassessment of previously identified [condition]. Prior complete fetal echocardiogram on [date] demonstrated [baseline]. Today, [2D / M-mode / color / spectral] evaluation demonstrated [findings]. Compared with the prior study, the condition is [interval change]."

Document, Separate, Verify

Keep the obstetric and cardiac stories distinct, then confirm the record supports the work

SAME-DAY OB ULTRASOUND + FETAL ECHO

- ☐ Both may be medically necessary on the same day.
- ☐ Use distinct indications, acquisition, and documentation.
- ☐ Growth scan: biometry, fluid, and the obstetric indication.
- ☐ Fetal echo: lesion, cardiac structure, function, hemodynamics, and rhythm.

Coding edits, modifiers, bundling, and coverage vary. Verify payer policy, current CPT guidance, institutional compliance rules, and applicable edits.

HARD STOPS

- No 76828 without stored spectral waveforms.
- Do not substitute a few still images for a fetal echo.
- Do not assume color supports spectral Doppler coding.
- Do not bury the cardiac study inside a generic growth report.
- Do not repeat images without answering the target question.
- Do not omit direct comparison with the prior study.

THE 10-SECOND CHECK BEFORE CLOSING

SONOGRAPHER: DO I HAVE...

- ☐ Cine loops?
- ☐ The lesion?
- ☐ Color?
- ☐ Spectral waveforms if indicated?
- ☐ Measurements?
- ☐ Function?
- ☐ Rhythm?

PHYSICIAN: DID I DOCUMENT...

- ☐ Specific indication?
- ☐ Prior complete echo?
- ☐ Today's actual components?
- ☐ Cardiac findings?
- ☐ Interval change?
- ☐ Plan / follow-up?
- ☐ Separate report when appropriate?



THE BOTTOM LINE

Follow-up fetal echo is a purposeful reassessment of fetal cardiovascular disease. The order, stored imaging, interpretation, comparison, documentation, and submitted codes must support one coherent clinical story.



Full deck + references

Key references: Moon-Grady AJ, et al. ASE Guidelines. J Am Soc Echocardiogr. 2023;36:679-723. doi:10.1016/j.echo.2023.04.014. Society for Maternal-Fetal Medicine, Required Components for Coding Fetal Echocardiograms. This tool is educational and is not a substitute for current CPT guidance, payer policy, institutional compliance review, or clinical judgment. CPT is a registered trademark of the American Medical Association.